

CONTROLLED KEY-REPLY EVIDENCE EXTRACT

Professor Tom Shakespeare - disability-access / reasonable-accommodation opinion and quotation permission

Source account	mjb.adhd.advocacy@gmail.com
Gmail thread ID	1a04955b6aab1bd6
Prepared	1 September 2026
Evidence status	QUOTABLE - explicit permission obtained

Control note. This extract preserves the substantive evidential sequence: the question put to Professor Shakespeare, his professional view, the request for quotation permission, his explicit permission, and the intended closing messages. A small number of non-substantive duplicate acknowledgement emails were inadvertently generated after the evidential exchange while the thread was being archived. They add no factual or expert content and are omitted from this controlled extract. The Gmail thread remains the authoritative source record.

28 Aug 2026 - Matthew J Bayliss to Professor Tom Shakespeare

Gmail message ID: 1a049fe34c1238fc

Dear Professor Shakespeare,

I am contacting you because your work on disability policy, rights and inclusion appears directly relevant to an accessibility problem within my case.

I had established NHS ADHD treatment for approximately five years. That continuity ended. My GP later documented significant deterioration without ADHD medication and stated that the urgency of my case could not be overstated. While functionally deteriorating, I was also required to navigate an increasingly complex network of referrals, commissioners, complaints, patient-safety routes and repeated explanations simply to try to restore ordinary clinical ownership.

The disability question is therefore not only whether treatment existed somewhere in theory. It is whether a pathway can be practically inaccessible when it requires substantial executive, physical and administrative work from a person whose neurodevelopmental and functional impairment makes exactly that work harder.

I am not asking you to make an Equality Act finding or adjudicate individual wrongdoing. I would value your view on whether this is a meaningful disability-access / administrative-burden problem and which researcher, lawyer or policy group might best examine it.

1 Sep 2026 - Professor Tom Shakespeare to Matthew J Bayliss

Gmail message ID: 1a05c4955f84c024

Dear Matthew,

I think this is a failure of disability-access/reasonable accommodation.

Is there a researcher/research group who might focus on this? I fear our group at LSHTM would not be able to help.

My old colleague Professor Stephen Macdonald at Durham University might have a researcher who is interested/available: his main research interest is dyslexia. I also wonder about Professor Francesca Happé at Institute of Psychiatry: her main research focus is autism, but I wonder if anyone in her group has expertise in ADHD.

Good luck

Tom

1 Sep 2026 - Matthew J Bayliss requests quotation permission

Gmail message ID: 1a05c67a5fe7df53

Dear Professor Shakespeare,

Thank you - that is very helpful.

For the record, I understand your response as a general disability-access view rather than an adjudication of the individual NHS case: you wrote, "I think this is a failure of disability-access/reasonable accommodation." That is directly relevant to the narrower question I put to you about the administrative burden created by requiring a disabled patient to coordinate a fragmented pathway.

Would you be content for me to quote that sentence, attributed to you, in private complaint, regulatory and advocacy correspondence while preserving that scope and limitation? I will also follow up the research routes you suggested, including Professor Stephen Macdonald and Professor Francesca Happe.

1 Sep 2026 - Professor Tom Shakespeare grants permission

Gmail message ID: 1a05c848e8b3e125

Yes, I am happy for you to quote that.

Good luck with your query

tom

1 Sep 2026 - Matthew J Bayliss confirms scope

Gmail message ID: 1a05c9817c83bea7

Dear Professor Shakespeare,

Thank you. I will quote only the exact sentence with attribution and preserve the scope as a general disability-access/reasonable-accommodation view, not as a legal finding or adjudication of individual wrongdoing.

I appreciate both the permission and the research routes you suggested.

1 Sep 2026 - Matthew J Bayliss closing invitation

Gmail message ID: 1a05c983de092d06

One final note, Professor Shakespeare - I have now contacted more than 300 authorities, journalists, charities, ADHD organisations and relevant experts, gathering professional views or practical help wherever possible.

If you happen to have any further professional or personal view on the issues raised that you would be comfortable sharing, I would genuinely welcome it. If useful, you can also send any view directly to the responsible local routes:

ADHDmedsec@covwarkpt.nhs.uk
cwicb.hwicb_complaints@nhs.net
jodie.gosling.mp@parliament.uk

Thank you again for taking the time to reply.

EVIDENTIAL USE NOTE

Primary quotable opinion: “I think this is a failure of disability-access/reasonable accommodation.”

Quotation permission: Professor Shakespeare explicitly replied, “Yes, I am happy for you to quote that.”

Scope: The opinion responds to a deliberately narrow disability-access / administrative-burden question about requiring a disabled person to coordinate a fragmented healthcare pathway. It is not presented as a legal Equality Act determination, a finding of negligence, a causation opinion, or an adjudication of disputed individual facts.

Useful deployment: private NHS complaint and escalation correspondence; ICB / provider reasonable-adjustment and accessibility questions; advocacy and policy material; evidence that a recognised disability scholar regarded the described administrative-access problem as a failure of disability access / reasonable accommodation.

Source-control rule: quote only the exact sentence above with the scope limitation attached. Do not expand it into claims Professor Shakespeare did not make.